



CASE STUDY

# Health & Wellness Initiative



# Contents

|    |                                       |
|----|---------------------------------------|
| 1  | <b>Executive Summary</b>              |
| 2  | <b>Introduction</b>                   |
| 3  | <b>Project Scope</b>                  |
| 4  | <b>Pilot Project Overview</b>         |
| 5  | <b>Strategic Framework</b>            |
| 6  | <b>Implementation Model</b>           |
| 7  | <b>Key Challenges Identified</b>      |
| 8  | <b>Outcomes &amp; Impact</b>          |
| 9  | <b>Insights &amp; Lessons Learned</b> |
| 10 | <b>Partnership Networks</b>           |
| 11 | <b>Future Outlook</b>                 |

# 01

## Executive Summary

Omer-e-Rawan, a Section 42 non-profit, launched Pakistan's first school-based health and wellness model across two public schools in Islamabad. In collaboration with the Ministry of Federal Education, the pilot screened over 1,600+ students, identifying 458+ undiagnosed physical issues and 40% with psychological concerns. On-campus wellness centers provided immediate care, one-on-one counseling, and referrals to hospitals, resulting in emotional improvement for 80% of flagged students and academic progress for 20%.

This pilot proved that delivering care within schools is not only feasible but transformative. Backed by the Ministry, the model is now scaling nationwide through institutional and corporate partnerships.

# 02

## Introduction

### About Omer e Rawan

In a country where mental health remains largely unspoken and healthcare access is fragmented, Omer-e-Rawan emerged as a transformative response to an invisible crisis. Officially registered under Section 42 with the Securities and Exchange Commission of Pakistan (SECP), Omer-e-Rawan is a non-profit initiative designed to offer early detection, awareness, and integrated care for physical and mental health, starting with Pakistan's most underserved population: school-going children.

Omer-e-Rawan operates with a belief that well-being is not a privilege, but a right. By embedding health and wellness centers directly into school systems and fostering collaborations with health institutions, the initiative is redefining how care is accessed, understood, and acted upon in early adolescence.



### Vision

To build a Pakistan where every child grows up mentally resilient and physically well, with equitable access to health and emotional support, regardless of background or socioeconomic status.



### Mission

Omer-e-Rawan aims to ensure early detection, timely care, and long-term support for students facing health or emotional challenges. By embedding wellness centers in schools, building partnerships, and fostering awareness, the initiative creates safe, stigma-free spaces where every child can access the physical and mental care they need to grow, learn, and thrive.

# 03

## Project Scope

The pilot program by Omer-e-Rawan marked a first-of-its-kind intervention within Islamabad's public school sector, in collaboration with the Ministry of Federal Education and Professional Training. The pilot was not a single-dimensional medical check-up, it was a multi-tiered social reform effort aimed at breaking stigma, enabling access, and empowering action.



## Awareness

- Hosted mental health literacy sessions across different age groups (primary to intermediate) to explain topics like stress, bullying, emotional self-regulation, and empathy.
- Engaged parents through events and onsite sessions—reducing fear, increasing consent, and building trust.
- Designed custom awareness campaigns using student art, banners, and storytelling to promote peer-to-peer advocacy.
- Trained teachers and school staff on recognizing behavioral red flags.



## Action



- Implemented in-school wellness centers at selected government institutions, fully equipped with physicians & mental health professionals.



- Created an integrated student database, mapping key indicators such as attendance, health status, family circumstances, & referral needs.



- Facilitated on-ground treatment referrals through hospitals & university clinics.



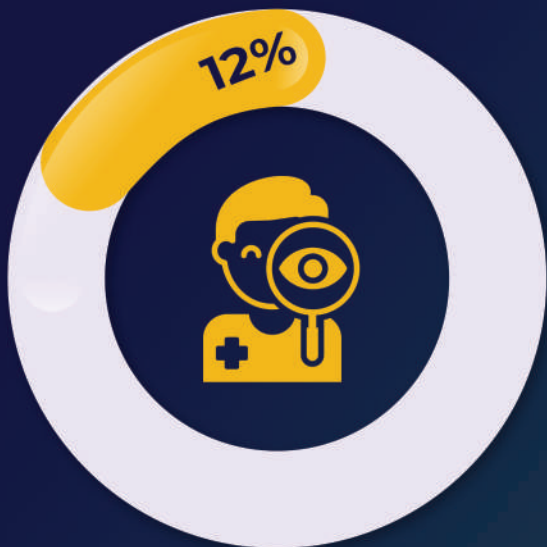
- Launched mass screening drives for mental, physical, & emotional health, reaching hundreds of students in the first phase.



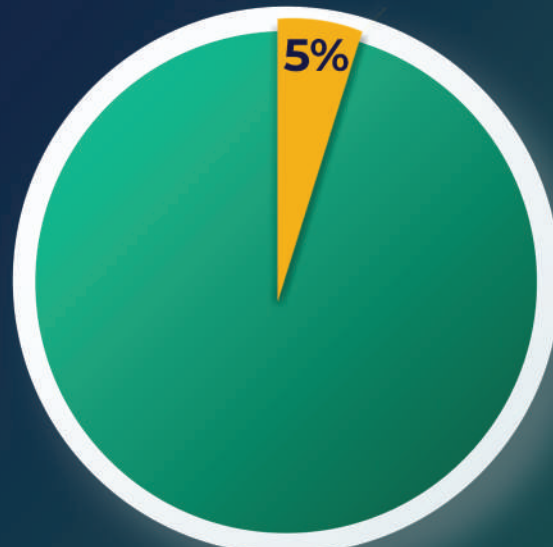
- Actively flagged students with issues such as low vision, undiagnosed skin infections, severe anxiety, & trauma symptoms.



## Student-Centric Impact



12% of students screened required corrective eye treatment, previously undiagnosed.



Over 5% identified as chronic absentees, with underlying causes ranging from untreated health issues to financial instability or school anxiety.



Psychological red flags were observed in **7%** of students, particularly those experiencing emotional neglect or parental separation.



Parents expressed overwhelming gratitude, many stating it was the first time they had received this kind of care for their children.

# 04

## Pilot Project Overview IMCG F-11/3 & ICG F-6/2, Islamabad

In its foundational phase, Omer-e-Rawan Health and Wellness Initiative implemented its first pilot across two public sector institutions in Islamabad:

1. Islamabad Model College for Girls (IMCG), F-11/3
2. Islamabad College for Girls (ICG), F-6/2

This multi-tiered intervention combined on-ground screenings, in-school wellness infrastructure, and extensive awareness programming, laying the groundwork for what a school-based care model could look like in Pakistan.

### IMCG F-11/3 – School-Based Wellness Center



The first-ever Omer-e-Rawan Wellness Center was established within the IMCG F-11/3 campus, equipped with:



A general physician



A clinical psychologist



A student counselor

A total of **600 - 625** students were evaluated during the first round of health & psychological screening.



The initial awareness sessions engaged students from multiple grades, covering topics like:



- Teachers were oriented to detect behavioral & emotional red flags, while parents were invited to participate in consent-based screening and treatment pathways actively.
- The center quickly evolved into a hub for academic counseling, emotional support, & physical care, helping bridge the health-education divide.

## ICG F-6/2 – Health Screening Drive & Partner Engagement



At ICG F-6/2, Omer-e-Rawan conducted a mass health & wellness screening, reaching **1027** students over multiple days.



Screenings were carried out in collaboration with:

- Final-year students from Rawalpindi Medical University.
- Medical & psychology students from Riphah International University, Fazaia Medical College, & NUST.
- Foundation University.
- Doctors from PAF Hospital.

## Health Parameters Checked

- Physical (height, weight, BMI).
- Vitals (blood pressure, heart rate).
- Eye exams.
- Sensory tests (taste, smell, hearing).
- Neurological tests (motor reflexes, balance).
- Lab tests (CBC, LFTs, RFTs).
- Exams by specialists (gynecologist, ENT, gastroenterologist, neurologist, dentist, dermatologist).
- Mental Health: mood assessments, behavioral observations, anxiety/depression screenings, stress level evaluations, social functioning checks.





A total of **110** students screened required further follow-up care, including **64** referrals to hospitals:

- Al-Shifa Eye Hospital
- PAF Hospital



**110 STUDENTS**



The event was attended by school leadership, Omer-e-Rawan board members, and Dr. Mowadat Hussain Rana, a distinguished psychiatrist and member of the OeR Board of Directors. He walked the on-ground team through the student screening protocols, underscoring the initiative's commitment to quality, empathy, and student-centered care.



**"When students feel good, they dream bigger & chase those dreams with confidence."**

**1,600+**

students screened across both schools.

**25+**

students received direct on-site medical support.



Health parameters checked included:



Vision



ENT



Nutrition



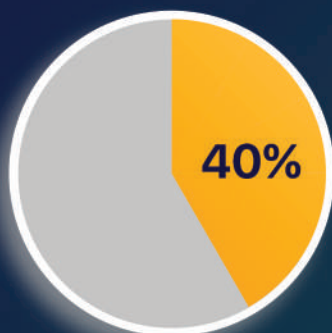
Hygiene



Physical Exams



Lab Tests



40% of students flagged for psychological or behavioral concerns.

60%

20%

60% of flagged students received in-school counseling; 20% referred externally.

**110**

students received additional care (glasses, ENT, dermatology, pediatric support).

**64**

students referred to partner hospitals.



Post-intervention assessments showed notable improvement in emotional, behavioral, &amp; academic outcomes.



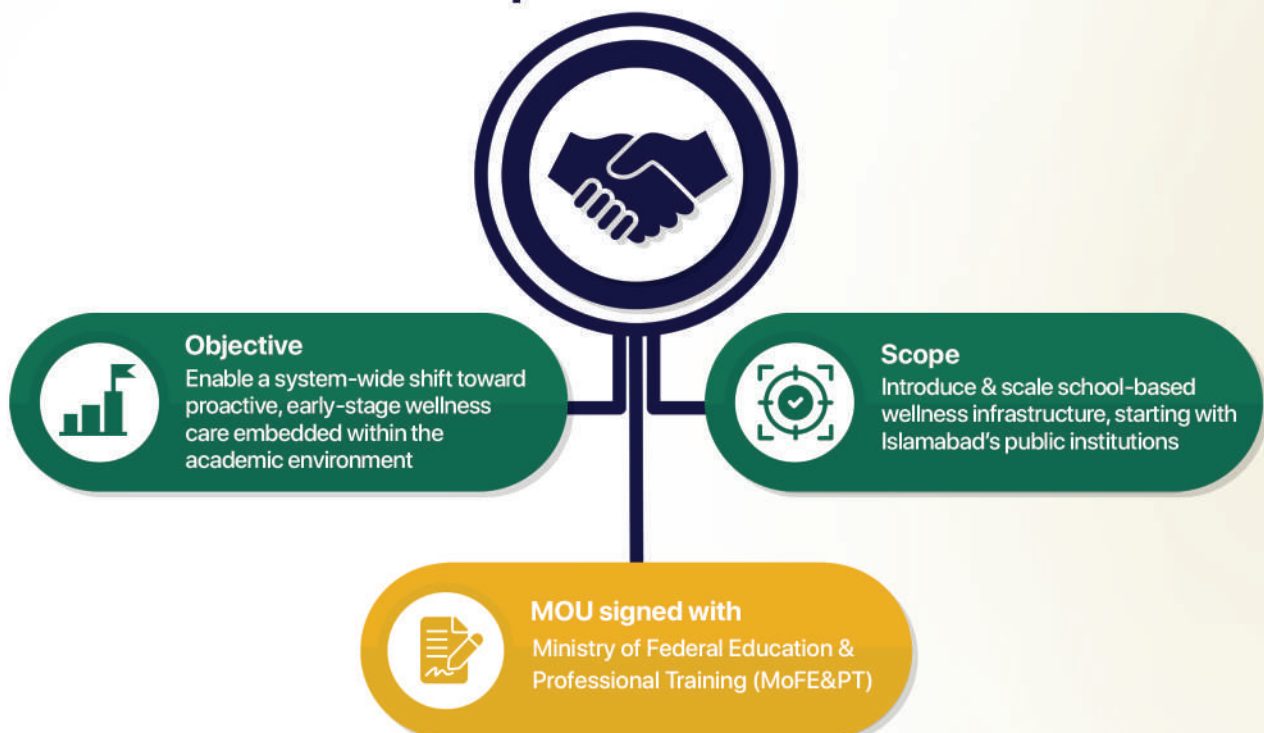
# 05

## Strategic Framework Building Wellness into the Federal Education System



A landmark step forward: Omer-e-Rawan signed an MoU with the Ministry of Federal Education & Professional Training (MoFE&PT) to bring structured health and wellness programs into the federal public school system. This partnership reinforces the belief that student well-being is not a privilege but a core part of education.

### Institutional Partnership



## School Selection & Assessment Criteria

Partner schools were selected through a collaborative evaluation process between Omer-e-Rawan and the Federal Directorate of Education (FDE), based on:



Geographical reach & student population



Infrastructure feasibility for setting up wellness rooms



Absenteeism rates, student behavioral indicators, & academic vulnerability



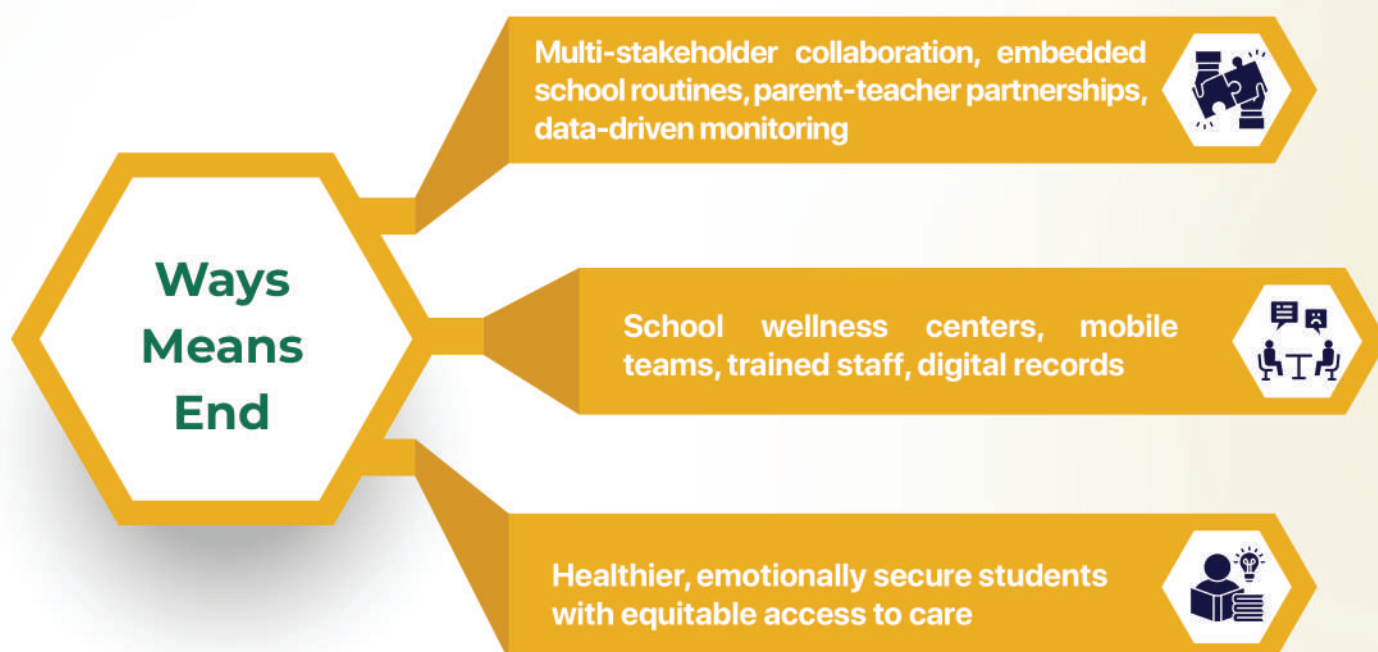
Willingness of school leadership to champion wellness integration

The first two selected institutions, **IMCG F-11/3** and **ICG F-6/2**, served as pilot models to assess feasibility, operational design, and community receptivity.



## Overall Thought Process & Strategic Design

Omer-e-Rawan's approach follows a "**ways-means-ends**" logic model:



# 06

## Implementation Model School-Based Wellness Infrastructure

Omer-e-Rawan follows a structured, replicable model that prioritizes school-wide engagement, targeted health actions, and integrated care pathways. Below is the operational framework:

### Wellness Center Flow

#### 1. School Selection

In collaboration with the Ministry, priority schools are chosen based on need, location, and infrastructure feasibility.

#### 2. Room Assignment

Each selected school allocates a room exclusively for the Wellness Center—visibly branded and emotionally safe.

#### 3. Baseline Student Data Mapping

- Enrollment records digitized.
- Basic demographics + health history documented.
- Student portfolios are created to track health, attendance, and academic performance.

#### 4. Initial Screening

- Conducted by a qualified doctor and psychologist.
- Covers physical health (skin, ENT, eyes) and mental health (behavioral screening tools).
- Students are assessed by class groups to ensure no one is missed.

#### 5. Flagging of Risk Cases

Students identified with medical or psychological concerns are categorized:

- Health Issues: Vision, dental, hygiene, and skin.
- Mental Wellness: Anxiety, anger issues, trauma exposure.
- Social Indicators: Absenteeism, economic neglect, family separation.

## 6. Consent and Parent-Teacher Engagement

- Consent forms sent home; follow-ups conducted via calls/meetings.
- Teachers provided reports to help understand behavioral patterns.
- Orientation sessions held for families to understand support pathways.

## 7. Specialist Interventions and Referrals

- In-school follow-up by GP or psychologist.
- Students referred to partner hospitals, university clinics, or external health camps when advanced care is needed.

## 8. Long-Term Monitoring

- Student records are updated after each intervention.
- Teacher and parent inputs are tracked for effectiveness.
- Critical cases are placed under continuous observation.

*This school-to-care model creates a loop of preventive screening, safe disclosure, timely treatment, and family involvement, ensuring long-term well-being outcomes.*



## 07

## Key Challenges Identified From Silence to Systemic Change

Despite the pilot's success, Omer-e-Rawan encountered deep-rooted barriers that reflect the broader state of health access in Pakistan.

### 1. Societal Stigma

Mental health remains a taboo in many communities. Parents often feared labels or believed counseling was "unnecessary," leading to delays in consent and cooperation.

### 3. Chronic Absenteeism & School Neglect

Students missing school were often misjudged as "irregular" or "disobedient." In reality, many faced emotional neglect, domestic violence, or lived without adequate adult supervision.

### 5. Consent & Coordination

Many students initially hesitated to open up. The absence of emotionally secure environments delays the reporting of abuse, anxiety, or school pressure.

### 2. Unaddressed Vision & Physical Health Issues

12% of students had unidentified eyesight problems, affecting academic performance & classroom confidence—most had never seen an optometrist before.

### 4. Lack of Safe Spaces

While the model was built around parental consent, follow-ups required persistent communication, especially in low-income families with limited phone access or high mobility.



These insights will shape the next phases of the program, which include more teacher training, peer support models, and community-led awareness campaigns.

# 08

## Outcomes & Impact

The Omer-e-Rawan Health and Wellness Center pilot program delivered meaningful change across both participating institutions—Islamabad Model College for Girls F-11/3 and Islamabad College for Girls F-6/2—within just a few months of implementation. The initiative was integrated into the school ecosystem with minimal disruption, ensuring that health screenings and emotional support services became a natural extension of the learning environment.

## Student Engagement & System Integration

By positioning the wellness center within the school premises, Omer-e-Rawan created a familiar, accessible space where students could openly discuss their health concerns. The constant presence of a doctor, psychologist, and counselor fostered trust among students, parents, and faculty, turning the center into a safe hub for motivation, academic counseling, and health support. The school faculty was regularly engaged in discussions and referrals, further embedding the program into daily school operations.



60%

### Screening Reach

Over 60% of students in the pilot schools underwent both physical & mental health screenings.



12%

### Early Detection

More than 12% of students were identified with undiagnosed vision issues, severely affecting their academic performance. Students were referred & treated in collaboration with partner institutions like Al-Shifa Trust Eye Hospital.



5%

### Absenteeism Reduction

More than 5% of students who had been chronically absent (over one month without attending) were re-engaged through family outreach & follow-ups, addressing underlying health & financial issues.

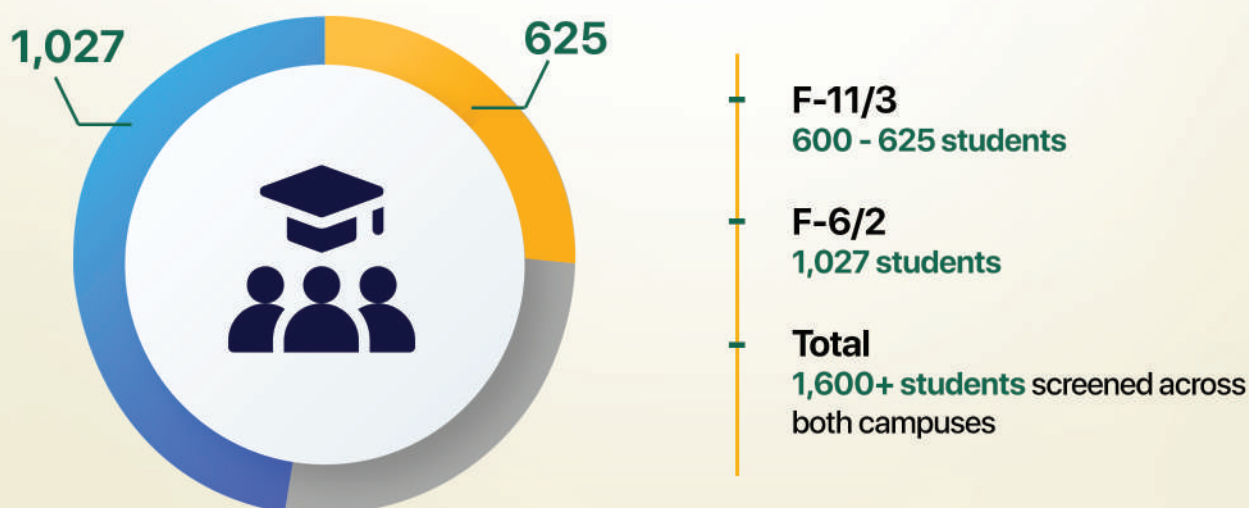


### Referrals & Treatment

Students requiring specialized care, including those with psychological needs or complex medical conditions, were referred to partner hospitals under signed MOUs—ensuring continued care beyond school-based intervention.



## Total Students Screened (Combined Physical & Mental Health)



## Health Parameters Assessed

BMI, vitals, eye and sensory tests, neurological and physical examinations, lab diagnostics, and specialist evaluations across 10+ medical disciplines (e.g., ENT, dermatology, pediatrics, gynecology).

## Undiagnosed Health Issues Identified

**458+**

students were found with undiagnosed medical concerns, many impacting academic performance.



## On-Site Medical Support Provided



25 students received direct treatment or prescriptions on campus



110 students received specific medical interventions:

- 39 prescription glasses
- 51 ophthalmology treatments
- 13 dermatology cases
- 5 pediatric, 2 ENT, & additional specialty care

## Referred to Partner Hospitals



**AL-SHIFA TRUST**

Al-Shifa Trust Eye Hospital



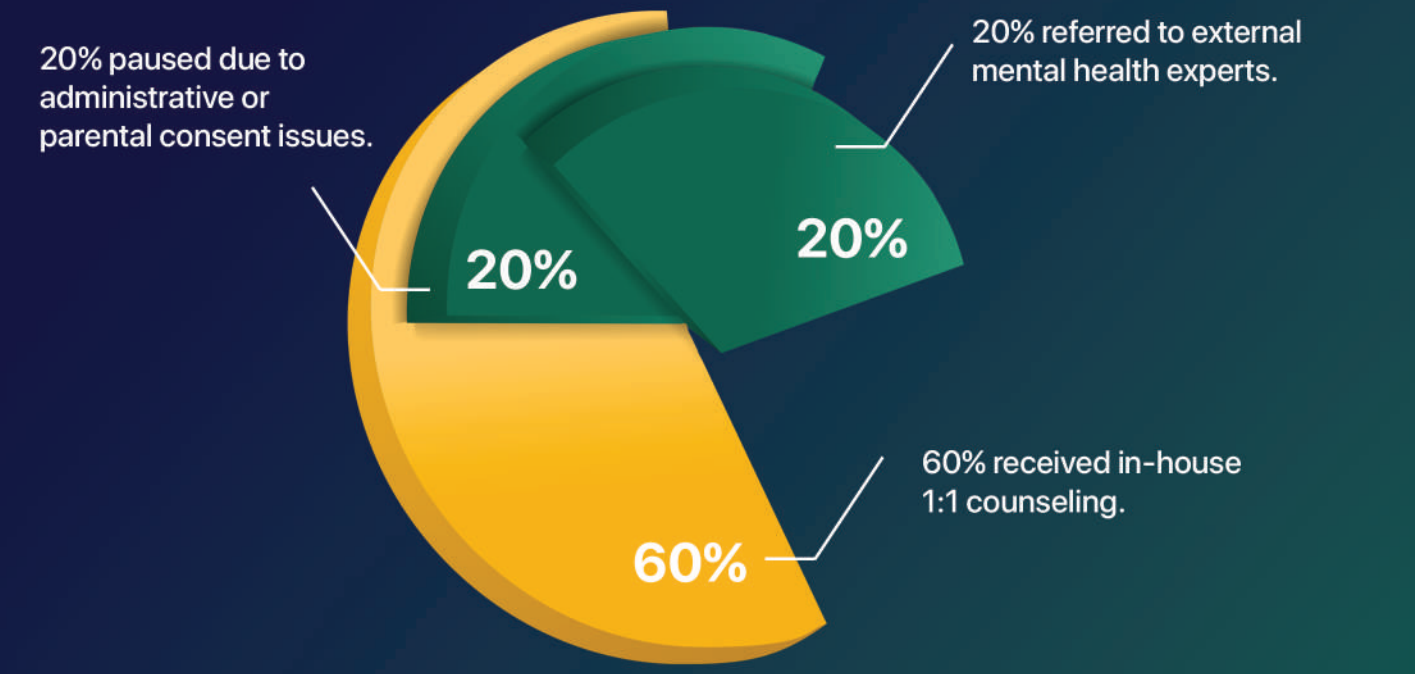
Polyclinic Hospital



PAF Hospital Unit 2

## Mental Health Flagging & Counseling

**40% of screened students** were flagged for emotional or psychological concerns.  
Of these:



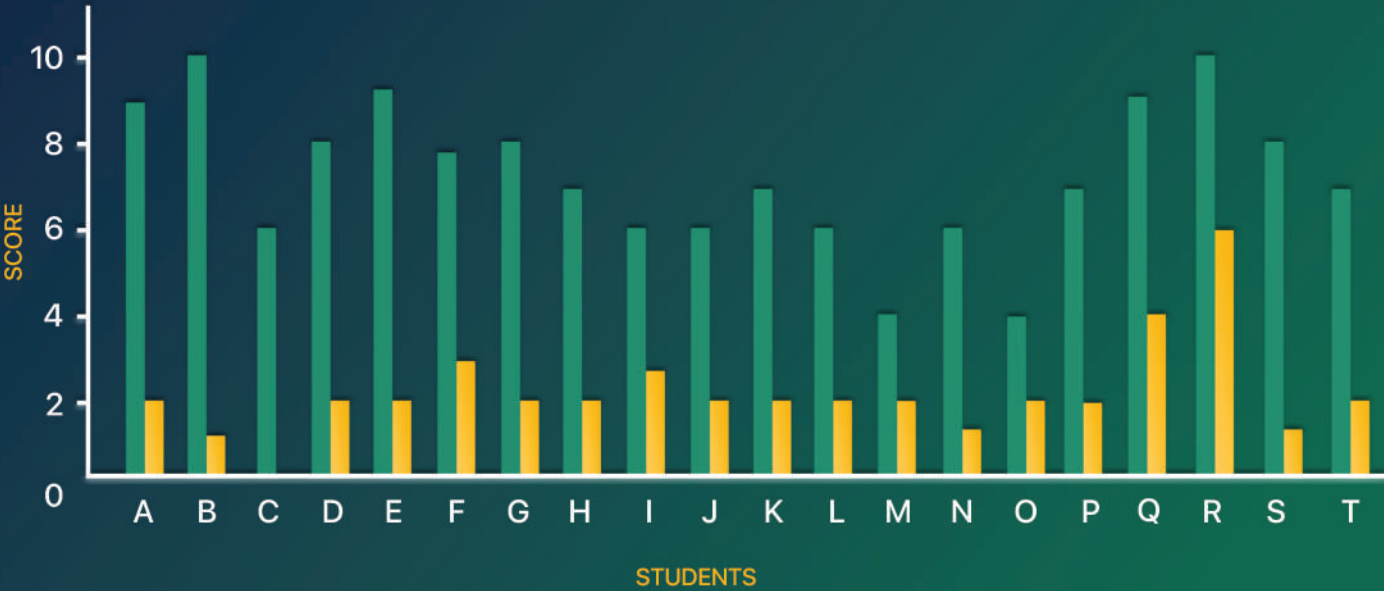
## Counseling Outcomes

**60% of cases** were successfully terminated post-therapy.

Statistics of improved students:

**80%** showed emotional & behavioral progress

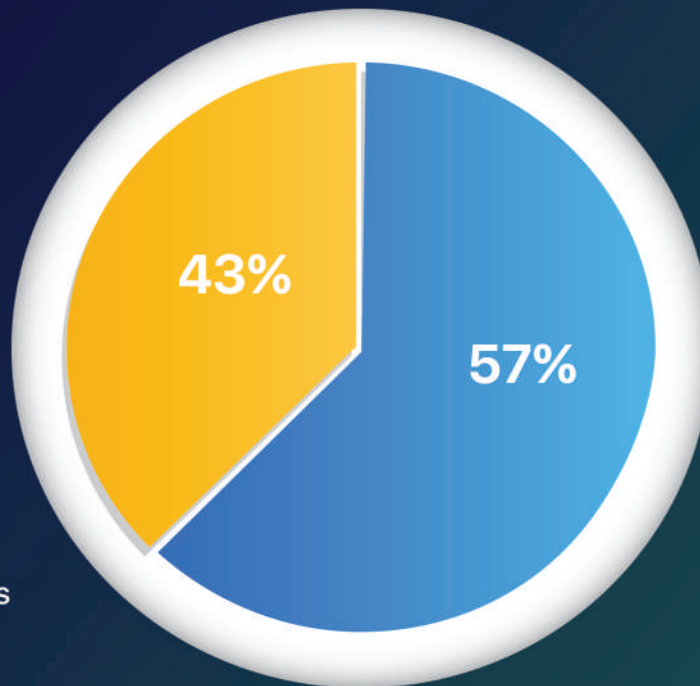
**20%** demonstrated academic improvement



## Terminated & Ongoing

Terminated students: 57%

Ongoing or other: 43%

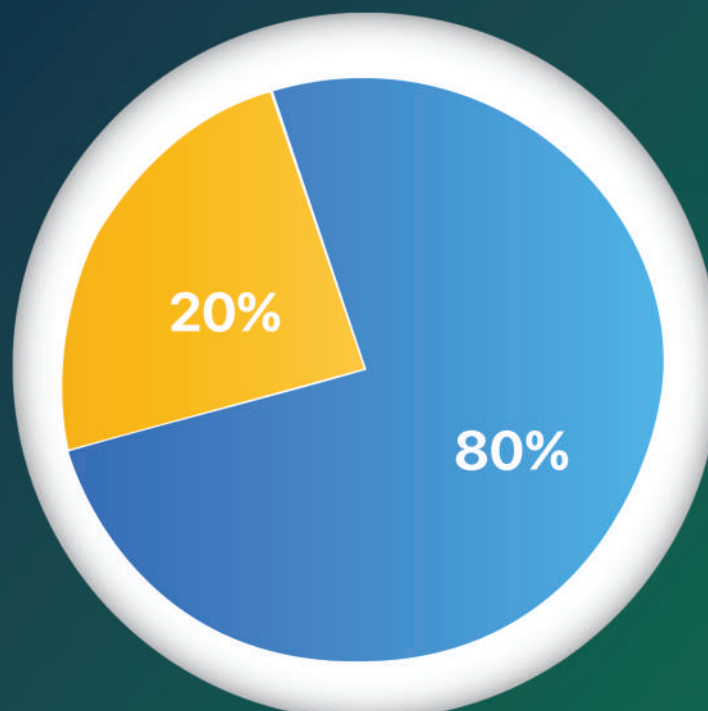


Therapy outcomes  
among flagged students

## Specific Progress

Academic: 20%

Emotional & behavioral: 80%



Type of improvement in  
students with progress

## Qualitative Outcomes

- **Strengthened Trust in Public Institutions**

The initiative renewed parental confidence in the public school system by showcasing a tangible commitment to student well-being. Parents not only consented to screenings but began participating in health education sessions as trust deepened.

- **Empowered Educators**

Teachers were trained to observe signs of distress and were encouraged to work in tandem with counselors, creating a more emotionally aware and responsive academic environment.

- **Community Ownership**

From Foundation University's volunteer students to active participation by teachers and school administrators, the pilot witnessed multi-level engagement that laid the groundwork for sustainable impact.



## Success Stories

### Vision Restored

- **Vision Restored:** Dozens of students identified with impaired eyesight received glasses, leading to improved focus and academic performance. Many were unaware of their condition prior to the screening.

### Critical Care Delivered

- **Critical Care Delivered:** One student flagged during routine health screening was found to have a serious cardiac condition and was referred for surgery—an intervention that may have saved her life.



Watch her story



These outcomes reflect the strong potential of school-based health and wellness models to address interlinked physical, emotional, and academic barriers. Through a data-backed approach, Omer-e-Rawan is not just offering treatment—it is redefining how care can be delivered where children learn and grow.

# 09

## Insights & Lessons Learned

### Physical Health Impacts the Mind

The pilot revealed a clear pattern: poor physical health was fueling mental distress. Many students faced malnutrition and untreated illnesses, which led to low focus, anxiety, and absenteeism. Once addressed, both emotional wellness and academic performance improved.

### Onsite Presence, Real Results

Having a dedicated wellness center within the school created more than convenience; it created trust. Students came forward, teachers referred concerns, and parents engaged. Daily access to doctors, counselors, and psychologists meant early action, not delayed intervention.



### Awareness Is a Group Effort

Our awareness sessions brought together students, teachers, and parents, each playing a vital role.

- Students learned about stress, bullying, and self-regulation.
- Teachers were trained to identify behavioral red flags.
- Parents received guidance, support, and reassurance.

## Change Happens When Everyone's Involved

Lasting impact came when everyone was part of the solution. When schools, families, and students worked together, awareness turned into action, and care became part of daily school life, not an external service.

# 10

## Partnership Network Building a Collective for Youth Wellness

Omer-e-Rawan's journey is not a solitary one. It is grounded in the belief that systemic change in student health and wellness requires collective ownership, guided by cross-sectoral expertise. Through its growing partnership network, Omer-e-Rawan has cultivated a robust ecosystem of support, collaboration, and impact-sharing that spans education, healthcare, policy, and corporate sectors.

### Strategic Partners from the Health, Education & Public Sector

#### Ministry of Federal Education & Professional Training

A pivotal partner in formalizing the initiative's entry into the federal education system. The Ministry's MoU with Omer-e-Rawan reflects a shared goal: to institutionalize structured wellness programs across public schools.



#### Rawalpindi Medical University

Contributed significantly through the volunteer efforts of final-year medical students, who supported on-ground screenings at Islamabad College for Girls (ICG), F-6/2. Their expertise helped flag critical physical health concerns and offered valuable guidance to students.



### Foundation University

Provided vital healthcare support during screenings at both pilot schools. Students from the university conducted physical assessments and actively engaged with youth, demonstrating a hands-on approach to wellness advocacy.



### Al-Shifa Trust Eye Hospital

Known for its specialized expertise, Al-Shifa added critical capacity in screening for undiagnosed vision issues, a recurring concern discovered during the pilot phase.

### Riphah International University

As an institutional ally, Riphah's presence supports the initiative's long-term academic and training ambitions, particularly in expanding access to mental health and therapeutic care in schools.



### Sybrid (Pvt.) Ltd.

A technology and operations partner, Sybrid offers digital infrastructure support that has aided in setting up scalable models for student data tracking, wellness center coordination, and reporting tools.

## Operational & Field-Level Collaborations (Pilot Project)

### PAF Hospital, Islamabad

Played a key role in on-site medical screenings. Doctors from the hospital conducted checkups and engaged students with compassion and care, reinforcing the initiative's goal of "connection beyond consultation."



### Fauji Fertilizer Company (FFC)

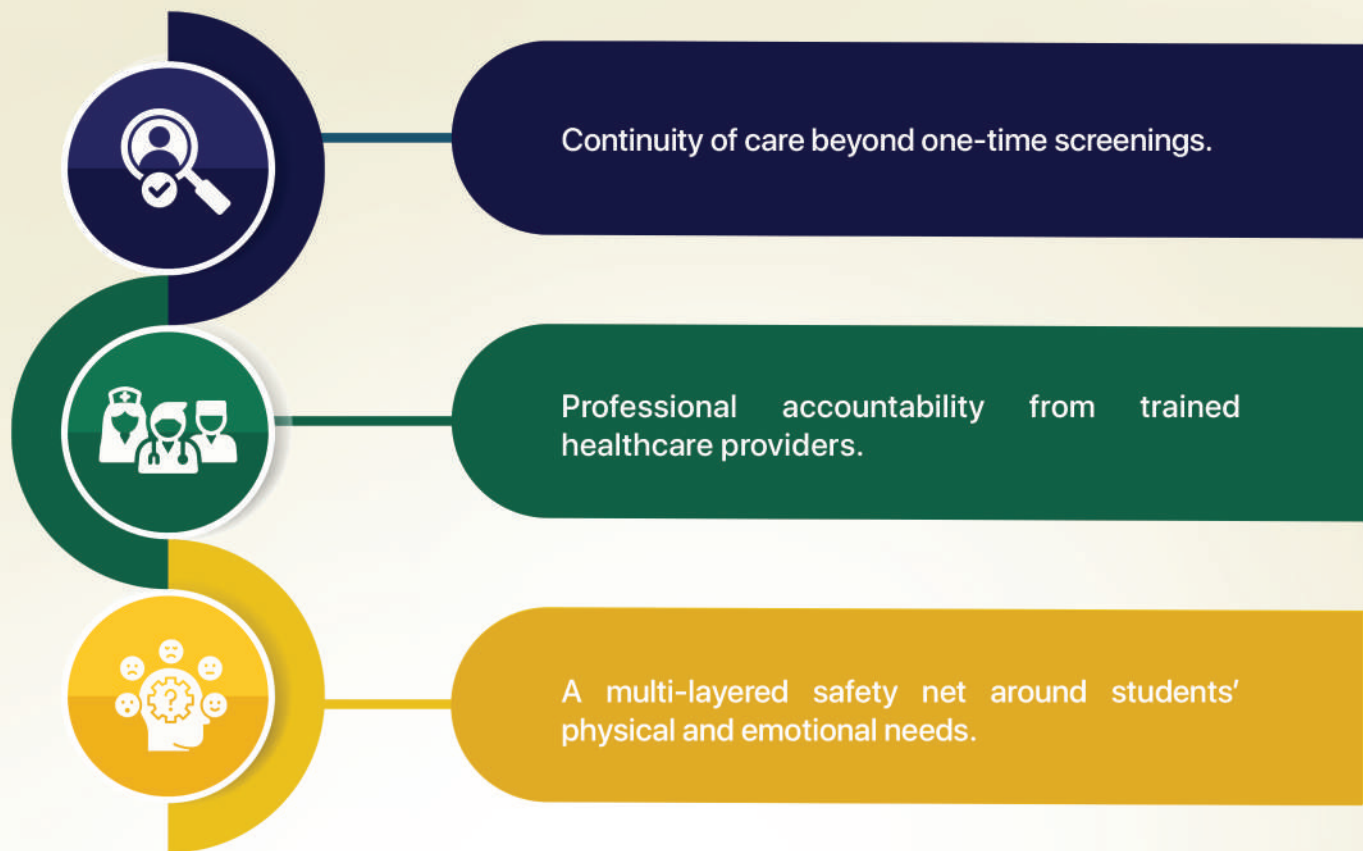
As a corporate supporter, FFC's team visited the wellness center to understand the ground impact. We also held a meeting with their leadership, including Jahangir Piracha, CEO of FFC, and Air Cdre (R) Farooq Bukhari, Head of CSR. Their interest in youth health initiatives reflects the growing corporate alignment with community wellness goals.

### School Leadership & Educators (Pilot Schools)

Heads and vice principals from Islamabad Model College for Girls (F-11/3) and Islamabad College for Girls (F-6/2) have actively supported implementation, participated in screenings, and ensured that wellness remains part of the school culture.



By grounding its work in trust-based partnerships, Omer-e-Rawan ensures:



As the initiative scales, these partnerships will serve as pillars for replication, enabling the establishment of more school-based wellness centers, expanding diagnostic access, and embedding emotional care into the mainstream education system.

# 11

## Future Outlook

The pilot phase proved that the model works. It is impactful, scalable, and urgently needed. Now, Omer-e-Rawan aims to expand its reach and deepen its impact, starting with underserved public schools in Islamabad and gradually expanding nationwide.

### School Expansion



**30+** schools & colleges in Islamabad over the next **3 years**, reaching **100,000+** children through **30 wellness centers**.

Expansion will follow the same model of in-school screening, care, & follow-up.

School selection will be data-driven, based on health gaps, infrastructure readiness, & student need.

### Centre of Excellence

- Omer-e-Rawan plans to launch a Centre of Excellence to standardize wellness protocols, research impact, and train school wellness teams.
- The center will also host innovation labs and serve as a central hub for future scale-up efforts.

## Strengthening Collaborations

Long-term partnerships with institutions like Riphah University, Rawalpindi Medical University, Fauji Foundation, Shifa Eye Trust Hospital, and NUST will support:



Human Resource & Volunteer Networks



Clinical Training



Referral-based Treatment Pathways

Ongoing collaboration with like-minded NGOs such as Rozan and Saving 9 will help integrate community mental health models and school safety training.

## Sustainability Through Fundraising

To ensure long-term impact, Omer-e-Rawan is committed to financial resilience:



Because every child deserves a safe space for care, inside the place where they learn. The next chapter is about scaling with stability and building a future where health and education go hand in hand.

